

Name : Mr. VISHAL GUPTA
Lab No. : 495105872
Ref By : dr p c agarwal
Collected : 17/1/2026 10:17:00AM
A/c Status : P
Collected at : GREEN PARK
J-4, Green Park Main, New Delhi - 110016
DELHI

Age : 24 Years
Gender : Male
Reported : 17/1/2026 1:12:18PM
Report Status : Final
Processed at : LPL-VASANT KUNJ LAB
NELSON MANDELA MARG, BUILDING No.1
L.S.C., SECTOR-B, POCKET-7, VASANT KU
NEW DELHI-110070



Test Report

Test Name	Results	Units	Bio. Ref. Interval
HEMOGRAM			
Hemoglobin (Photometry)	13.20	g/dL	13.00 - 17.00
Packed Cell Volume (PCV) (Calculated)	38.90	%	40.00 - 50.00
RBC Count (Electrical impedance)	4.64	mill/mm3	4.50 - 5.50
MCV (Electrical impedance)	83.80	fL	83.00 - 101.00
Mentzer Index (Calculated)	18.1		
MCH (Calculated)	28.40	pg	27.00 - 32.00
MCHC (Calculated)	33.90	g/dL	31.50 - 34.50
Red Cell Distribution Width (RDW) (Electrical Impedance)	14.60	%	11.60 - 14.00
Total Leukocyte Count (TLC) (Electrical Impedance)	6.00	thou/mm3	4.00 - 10.00
Differential Leucocyte Count (DLC)			
Segmented Neutrophils (Electrical impedance)	52.40	%	40.00 - 80.00
Lymphocytes (VCS) Technology)	38.50	%	20.00 - 40.00
Monocytes (VCS) Technology)	6.10	%	2.00 - 10.00
Eosinophils (VCS) Technology)	2.70	%	1.00 - 6.00
Basophils (VCS) Technology)	0.30	%	<2.00
Absolute Leucocyte Count			
Neutrophils (Calculated)	3.14	thou/mm3	2.00 - 7.00
Lymphocytes (Calculated)	2.31	thou/mm3	1.00 - 3.00
Monocytes (Calculated)	0.37	thou/mm3	0.20 - 1.00
Eosinophils (Calculated)	0.16	thou/mm3	0.02 - 0.50



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Test Name	Results	Units	Bio. Ref. Interval
Basophils (Calculated)	0.02	thou/mm3	0.02 - 0.10
Platelet Count (Electrical impedance)	153	thou/mm3	150.00 - 410.00
Mean Platelet Volume (Electrical impedance)	11.3	fL	6.5 - 12.0
E.S.R. (Capillary photometry)	2	mm/hr	0 - 15

Comment

In anaemic conditions Mentzer index is used to differentiate Iron Deficiency Anaemia from Beta- Thalassemia trait. If Mentzer Index value is >13, there is probability of Iron Deficiency Anaemia. A value <13 indicates likelihood of Beta- Thalassemia trait and Hb HPLC is advised to rule out the Thalassemia trait.

Note

- As per the recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood

- Test conducted on EDTA whole blood

anisocytosis +,
Predominantly Normocytic normochromic RBCs
WBC is normal in number.
Platelets are adequate.



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
LIVER & KIDNEY FUNCTION TEST (LFT & KFT)			
Creatinine (Modified Jaffe,Kinetic)	0.80	mg/dL	0.70 - 1.30
GFR Estimated (CKD EPI Equation 2021)	126	mL/min/1.73m2	>59
GFR Category (KDIGO Guideline 2012)	G1		
Urea (Urease UV)	32.36	mg/dL	13.00 - 43.00
Urea Nitrogen Blood (Calculated)	15.11	mg/dL	6.00 - 20.00
BUN/Creatinine Ratio (Calculated)	19		
Uric Acid (Uricase)	7.80	mg/dL	3.50 - 7.20
AST (SGOT) (IFCC,without P5P)	50.0	U/L	15.00 - 40.00
ALT (SGPT) (IFCC,without P5P)	99.0	U/L	10.00 - 49.00
AST:ALT Ratio (Calculated)	0.51		<1.00
GGTP (IFCC,L-y-glutamyl-3-carboxy-4-nitroanilide)	35.0	U/L	0 - 73
Alkaline Phosphatase (ALP) (IFCC-AMP)	81.00	U/L	30.00 - 120.00
Bilirubin Total (Oxidation)	1.37	mg/dL	0.30 - 1.20
Bilirubin Direct (Oxidation)	0.47	mg/dL	<0.3
Bilirubin Indirect (Calculated)	0.90	mg/dL	<1.10
Total Protein (Biuret)	7.10	g/dL	5.70 - 8.20
Albumin (BCG)	4.70	g/dL	3.20 - 4.80
Globulin(Calculated)	2.40	gm/dL	2.0 - 3.5
A : G Ratio (Calculated)	1.96		0.90 - 2.00



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
Calcium, Total (Arsenazo III)	9.49	mg/dL	8.70 - 10.40
Phosphorus (Phosphomolybdate UV)	3.50	mg/dL	2.40 - 5.10
Sodium (Indirect ISE)	139.50	mEq/L	136.00 - 145.00
Potassium (Indirect ISE)	4.33	mEq/L	3.50 - 5.10
Chloride (Indirect ISE)	103.10	mEq/L	98.00 - 107.00

Note

1. Estimated GFR (eGFR) calculated using the 2021 CKD-EPI creatinine equation and GFR Category reported as per KDIGO guideline 2012.
2. eGFR category G1 or G2 does not fulfil the criteria for CKD, in the absence of evidence of kidney damage
3. The BUN-to-creatinine ratio is used to differentiate prerenal and postrenal azotemia from renal azotemia. Because of considerable variability, it should be used only as a rough guide. Normally, the BUN/creatinine ratio is about 10:1
4. Test conducted in Serum



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
IMMUNOGLOBULIN IgE (CLIA)	272.90	IU/mL	0.00 - 158.00
Result Rechecked, Please Correlate Clinically.			

Note

1. Normal levels of IgE do not rule out possibility of IgE dependent allergies as the diagnostic sensitivity of the test depends upon elapsed time between exposure to an allergen and testing, patient age and affected target organs.
2. No close correlation has been demonstrated between severity of allergic reaction and IgE levels.
3. Test conducted on serum

TSH (THYROID STIMULATING HORMONE) ULTRASENSITIVE, SERUM (CLIA)	2.104	μIU/mL	0.550 - 4.780
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Note

1. TSH levels are subject to circadian variation, reaching peak levels between 2 - 4.a.m. and at a minimum between 6-10 pm . The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations.
2. Values <0.03 μIU/mL need to be clinically correlated due to presence of a rare TSH variant in some individuals.
3. Transient increase in TSH levels or abnormal TSH levels can be seen in various nonthyroidal diseases. Simultaneous measurement of TSH with free T4 is useful in evaluating the differential diagnosis



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Test Report

Test Name	Results	Units	Bio. Ref. Interval
HbA1c (HPLC, NGSP certified)			
HbA1c	5.6	%	4.00 - 5.60
Estimated average glucose (eAG)	114	mg/dL	

Interpretation

HbA1c result is suggestive of non diabetic adults (≥ 18 years)/ well controlled Diabetes in a known Diabetic

Interpretation as per American Diabetes Association (ADA) Guidelines

Reference Group	Non diabetic adults ≥ 18 years	At risk (Prediabetes)	Diagnosing Diabetes	Therapeutic goals for glycemic control
HbA1c in %	4.0-5.6	5.7-6.4	≥ 6.5	< 7.0

Note

1. Presence of Hemoglobin variants and/or conditions that affect red cell turnover must be considered, particularly when the HbA1C result does not correlate with the patient's blood glucose levels.
2. Test conducted in EDTA Whole blood

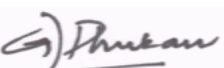
FACTORS THAT INTERFERE WITH HbA1C MEASUREMENT	FACTORS THAT AFFECT INTERPRETATION OF HbA1C RESULTS
Hemoglobin variants, elevated fetal hemoglobin (HbF) and chemically modified derivatives of hemoglobin (e.g. carbamylated Hb in patients with renal failure) can affect the accuracy of HbA1c measurements	Any condition that shortens erythrocyte survival or decreases mean erythrocyte age (e.g., recovery from acute blood loss, hemolytic anemia, HbSS, HbCC, and HbSC) will falsely lower HbA1c test results regardless of the assay method used. Iron deficiency anemia is associated with higher HbA1c


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-----End of report-----



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IMPORTANT INSTRUCTIONS

•Test results released pertain to the specimen submitted. •All test results are dependent on the quality of the sample received by the Laboratory. •Laboratory investigations are only a tool to facilitate in arriving at a diagnosis and should be clinically correlated by the Referring Physician. •Report delivery may be delayed due to unforeseen circumstances. Inconvenience is regretted. •Certain tests may require further testing at additional cost for derivation of exact value. Kindly submit request within 72 hours post reporting. •Test results may show interlaboratory variations. •The Courts/Forum at Delhi shall have exclusive jurisdiction in all disputes/claims concerning the test(s) & or results of test(s). •Test results are not valid for medico legal purposes. •This is computer generated medical diagnostic report that has been validated by Authorized Medical Practitioner/Doctor. •The report does not need physical signature.

(#) Sample drawn from outside source.

If Test results are alarming or unexpected, client is advised to contact the Customer Care immediately for possible remedial action.

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